

Pediatrics

1. A four-year-old child was brought to a health center with a complaint of cough of six days duration. On physical examination, the clinician noticed that the child had grade-II oedema of both feet. What is the appropriate intervention of this child? (Hidar 2012/ December 2019).

A. Give RUTF and Amoxicillin
B. **Refer the child to a hospital**
C. Give Amoxicillin and folic acid
D. Link to appropriate feeding program

2. A four-year-old child was brought to a health institution with a complaint of cough for three weeks duration. On history, the child had fever and night sweats. Gene X-pert result suggested PTB positive.

What is the other mandatory screening to be done for this child? (Hidar 2012/ December 2019)

A. Assess for anemia
B. Assess for diarrhea
C. Assess for dehydration
D. **Assess for acute malnutrition**

3. A two week-old neonate was brought to under-5 OPD with a complaint of umbilical ulceration. On examination, the neonate's umbilicus had infection.

What is the most appropriate management of this baby? (Hidar 2012/ December 2019)

A. Refer the infant to hospital immediately
B. **Pain umbilicus with 0.5% GV twice daily.....IMNCI #19**
C. Assure the mother that will disappear after some days
D. Give first dose of intramuscular Ampicillin and Gentamycin

4. A three-month-old baby was brought to a health center for vaccination. His mother verbalized that he had no history of vaccination.

Which vaccines should he given for this infant? (Hidar 2012/ December 2019)

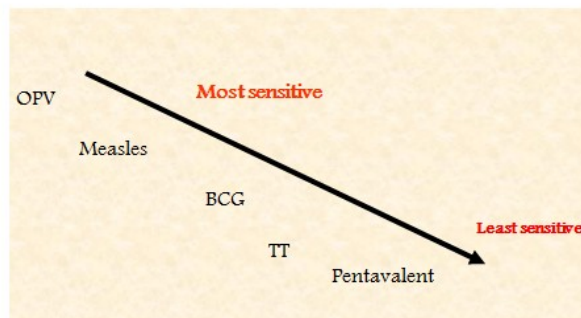
A. BCG, penta-1, PCV-1, OPV-0
B. BCG, penta-1, PCV-1, OPV-0, Rota-1
C. **BCG, penta-1, PCV-1, OPV-1, Rota-1**
D. BCG, penta-1, PCV-1, OPV-1, Rota-1, IPV

5. A clinician who is working in EPI unit of a health center forgot to substitute the source of power for vaccine containing refrigerator by a kerosene generator when electric power was not working.

Which vaccine would be damaged earlier? (Hidar 2012/ December 2019)

- A. **OPV**
- B. BCG
- C. Measles
- D. Rota vaccine

Vaccines sensitive to heat



6. A mother brought her 10-week-old baby to health center for second round vaccination. The mother lost the infant vaccination card. When the clinician checked registration log, he cannot get any information recorded about the infant. On history, the mother told him as one of the vaccines which is given on his right thigh is missed last time.

What is the missed vaccine for this baby? (Hidar 2012/ December 2019)

- A. IPV
- B. BCG
- C. **PCV**
- D. Pentavalent

Schedule ,Type, Dose Route of administration of vaccines

At birth	Diseases	Dose	Rout of adm
BCG	TB	0.05ml	ID Rt deltoid
OPV-0	Polio	2drops	Oral
Hep BD	Hepatitis B		Right (outer) mid-thigh)
At 6wks			
OPV -1	Polio	2 drops	Oral
Rota 1	Rotavirus		
Penta 1	Hib disease Hepatitis B Diphtheria Tetanus Whooping cough	0.5 ml	IM thigh
PCV1	Bacterial infe (otitis media, sinusitis and pneumonia, meningitis)	0.5 ml	IM thigh

Schedule ,Type, of vaccines...cont'd

At 14 th wks			
	Diseases	Dose	Rout of adm
IPV	Polio	0.5ml	IM right thigh
Penta 3	Hib disease Hepatitis B Diphtheria Tetanus Whooping cough	0.5 ml	IM left thigh
PCV3	Bacterial infe (otitis media, sinusitis and pneumonia, meningitis)		IM right thigh

NB: DPT, HiB and HBV replace by **Pentavalent** as single dose.

At 9 and 15 months measles Sc

7. A clinician checked the vaccine vial monitor (VVM) of a vaccine to decide whether a vaccine can be used or not. So, while checking the VVM, the color of the inner square was the same with the outer circle.

What should be the decision of the clinician? (Hidar 2012/ December 2019)

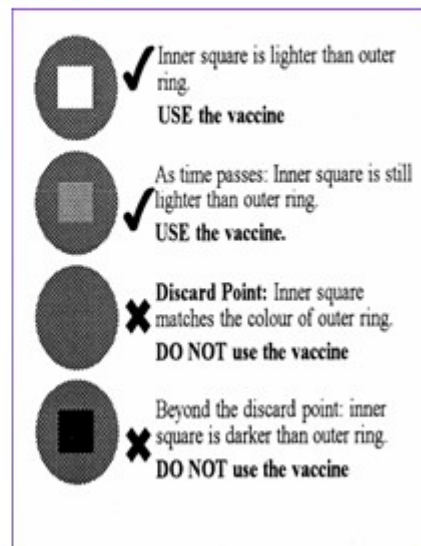
- A. Use the vaccine until inner square turns to dark
- B. **Discard the vaccine since it has been exposed to heat**
- C. Discard the vaccine since it has been exposed to freezing
- D. Perform shake test and pass the decision based on the result

Reading a Vaccine Vial Monitor



29/01/2025

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8. A three-day old 2.8kg neonate is on express breast feeding with amount of 100ml per day related to limited milk production of the mother. The physician ordered a maintenance fluid to compensate the remaining daily requirement.

What is the desired quantity to compensate the remaining daily requirement? (Hidar 2012/ December 2019)

- A. 336ml
- B. 236ml
- C. 252ml
- D. 152ml

Table 2.7: Day Wise Fluid Requirements for Newborn

Day	Fluid in ml/kg/day
1	80ml
2	100 ml
3	120 ml
4	135 ml
5	150 ml

Table 17:2 Estimated maintenance fluid requirements based on weight and postnatal age

Birth weight	Fluid rate (ml/kg/day)			
	Postnatal age			
	Day 1	Day 2	Day 3-6	≥ Day 7
<750 gm	100-140	120-160	140-200	140-160
750-1,000 gm	100-120	120-140	130-180	140-160
1,000 - 1,500 gm	80-100	100-120	120-160	150
1,500 - <2,500 gm	60-80	80-100	120-160	150
≥2,500 gm	60-80	80-100	90-150	

9. A 5.5kg neonate delivered at gestation age of 37weeks from a mother with high blood glucose level. At birth the neonate's blood glucose level was 32mg/dl, but it has been raised to more than 40mg/dl after single breast feeding.

What is the most likely cause of the hypoglycemia of the neonate? (Hidar 2012/ December 2019)

- A. Crossed maternal insulin inducing hyperinsulinism
- B. **Crossed maternal glucose induced hyperinsulinism**
- C. Congenital pancreatic anomalies related hyperinsulinism
- D. Congenital hepatic anomalies related hyperinsulinism

Crossed Maternal Glucose Induced Hyperinsulinism refers to a condition where a fetus produces excess insulin due to high levels of glucose in the mother's blood.

Mechanisms:

- When a pregnant woman has elevated blood glucose levels (often seen in gestational diabetes or pre-existing diabetes), this increases the glucose available to the fetus.
- The fetus, unable to metabolize glucose efficiently on its own, will respond by producing more insulin to manage the incoming glucose.
- After delivery, when the umbilical cord is cut, the supply of maternal glucose stops. However, the neonate may still have high levels of insulin circulating in its system, which can lead to rapid drops in blood glucose levels (hypoglycemia) because the insulin is still driving glucose into the cells, but there's no longer an adequate glucose supply.

10. A 12-day-old male neonate admitted in neonatal intensive care unit for the treatment of respiratory distress syndrome and he is under oxygen administration at 3L/min. on physical finding, RR=58/min, T°=37.6°C, OS=70%.

What is the priority actual nursing diagnosis for this child? (Hidar 2012/ December 2019)

- A. Increase body temperature
 - B. **Impaired gas exchange**
 - C. Ineffective breathing pattern
 - D. Difficult air way clearance
- **Impaired Gas Exchange:**
 - This diagnosis emphasizes the outcome of the respiratory process, specifically the ability to transfer oxygen and carbon dioxide effectively.
 - It indicates that even if the breathing pattern is adequate, there may still be problems with oxygenation (as evidenced by low oxygen saturation).
 - **Ineffective Breathing Pattern:**
 - This diagnosis focuses on the mechanics of breathing, such as irregular or inadequate breaths, potentially due to fatigue, obstruction, or other factors.
 - It addresses how the neonate is breathing rather than the result of that breathing.

11. An 18-day-old neonate admitted in neonatal intensive care units for the treatment of neonatal tetanus with a clinical feature of unable to breast feeding, spasm, stiffness, accumulation of oral secretion and seizure.

What is the most likely priority nursing intervention for the neonate? (Hidar 2012/ December 2019)

- A. Administration of diazepam to the spasm
 - B. Insert NG-tube for express breast feeding
 - C. Secure IV line for maintenance fluid
 - D. Suction the secretion to prevent aspiration
12. An immediate new born was presented with clinical finding of defect in closure of the posterior vertebral arches, typically involving L5 and S1. The meninges, CSF and spinal cord are in their normal alignment.
- What is the most likely maternal deficiency associated with the new born manifestation? (Hidar 2012/ December 2019)
- A. Thiamine
 - B. Pyridoxine
 - C. Folic acid
 - D. Ascorbic acid

13. A four-month-old infant was brought to under-five OPD with complaint of failure to increase body weight. On examination, the infant has **no edema**, WFL < -3 Z score value and visible wasting. What intervention should be avoided to this infant? (Hidar 2012/ December 2019)
- A. Amoxicillin
 - B. **Provide F-75**
 - C. Provide Vitamine A
 - D. Provide Folic acid
14. A four-year-old child brought to pediatric OPD with complaint of vomiting everything for one day duration. On examination, the child's conjunctiva and some part of the palm looked pale, WHL < -3 Z score, has edema, fatigue, tachypnea, slow capillary refilling time and was conscious. What is the mostly likely complication of the child? (Hidar 2012/ December 2019)
- A. SAM plus shock
 - B. **SAM plus Anemia**
 - C. SAM plus Stroke
 - D. SAM plus Dehydration
15. A four-year-old child was brought to pediatrics OPD with complaint of vomiting everything for one day duration. On examination, the child has cold hands, WFL < -3 Z score, capillary refilling time > 3 seconds and was unconscious. The nurse infuses IV fluid at 15ml/kg over 1 hour and the child fails to improve. What is the next intervention to this child? (Hidar 2012/ December 2019).....**75p**
- A. Provide 3-10 ml/kg ReSoMal with F-75
 - B. Provide nasogastric rehydration with ReSoMal
 - C. **Give 4 ml/kg of maintenance fluid for 1 hour**
 - D. Transfuse 10 ml/kg whole fresh blood over 3 hour
16. A two years old child was brought to pediatrics OPD with compliant of vomiting everything for one day duration. On examination, the child has cold hands, WFH < -3 Z score, fatigue, slow capillary refilling time and was unconscious but has no edema. What is the first priority actual nursing diagnosis for this child? (Hidar 2012/ December 2019)
- A. Activity intolerance
 - B. **Fluid volume deficit**
 - C. Impaired gas exchange
 - D. Altered tissue perfusion

17. A three-years-old HIV positive child was brought to ART clinic with complaint of difficulty of swallowing for two days duration. On assessment, the child is not adhere and has severe immunodeficiency and more than viral load of 1000ml/copies.

What is the best intervention for this child? (Hidar 2012/ December 2019)

- A. Switch to the 2nd line regimen
 - B. Continue to the current regimen
 - C. Repeat viral load test after 3months
 - D. Continue to the current regimen and antibiotics
18. A 12-year-old child was brought to a TB clinic with complaint of unable to hear sounds for two days duration. The child started anti-TB medication 2 month back. On examination, the child has itching sensation, deafness and dizziness.

What is the most likely cause of this problem? (Hidar 2012/ December 2019)

- A. Isoniazid
- B. Rifampicin
- C. Pyrazinamide
- D. Streptomycin

Common Side Effects

- **Nausea and Vomiting:** Gastrointestinal disturbances can occur.
- **Diarrhea:** May cause loose stools.
- **Loss of Appetite:** Decreased interest in food.
- **Fever:** Some patients may develop a mild fever.

Serious Side Effects

- **Ototoxicity:** Damage to the inner ear, which can lead to hearing loss or balance issues.
 - **Nephrotoxicity:** Kidney damage, which may manifest as changes in urine output or blood urea nitrogen (BUN) and creatinine levels.
 - **Allergic Reactions:** Rash, itching, or severe allergic reactions (anaphylaxis).
 - **Neuromuscular Blockade:** Can cause muscle weakness or respiratory depression, particularly in patients with pre-existing neuromuscular conditions.
 - **Tinnitus:** Ringing in the ears, which may be temporary or permanent.
19. A 9-month-old infant was brought to a hospital with difficult breathing and cough of one day. The infant had recurrent cough and born from a mother that had history of frequent atopic dermatitis.

What is the most pertinent clinical sign that can complete the diagnosis of this neonate? (Hidar 2012/ December 2019)

- A. Stridor
- B. Crackles
- C. Wheezing
- D. Crepitation

20. A 7-year-old known cardiac patient was brought to a hospital for routine follow up. On examination, the child has central cyanosis Hgb: 9mg/dl and HCT: 70%.

What is the most appropriate management of this child? (Hidar 2012/ December 2019)

- A. Blood transfusion
- B. Iron supplement
- C. Nutritional advice
- D. Performing phlebotomy

21. A five-year-old child was presented with swelling of both legs of three days duration. The child has history of skin rash three weeks back. On examination, the child blood pressure of 150/100mmHg.

What is the most likely diagnosis of the child's problem? (Hidar 2012/ December 2019)

- A. Malnutrition
 - B. Heart failure
 - C. Nephrotic syndrome
 - D. **Acute glomerulonephritis**
22. A three-year-old female child was presented with unconsciousness of three hours duration. On examination, the child has snoring, drooling secretion and respiratory rate of 55 breath/min.

What is the most likely actual nursing diagnosis of this child? (Hidar 2012/ December 2019)

- A. Altered breathing
 - B. Impaired gas exchange
 - C. **Ineffective airway clearance**
 - D. Ineffective breathing pattern
23. An eight-year-old child presented with breathing difficulty and leg edema of three days duration. On examination, the child has pallor, cyanosis, oxygen saturation of 86% and Hgb: 8 mg/dl.

What is the priority nursing intervention of this child? (Hidar 2012/ December 2019)

- A. Iron supplement
 - B. **Oxygen administration**
 - C. Anti-diuretic administration
 - D. Blood transfusion
24. A nine-year-old female child presented with a complaint of urinary urgency and frequency of five days duration. The child has family history of diabetes and has no pain.

What is the primary nursing intervention for this child? (Hidar 2012/ December 2019)

- A. Counsel the child to get antibiotic
- B. Counsel the child to drink more fluid
- C. Link the child to diabetic rehabilitation center
- D. **Counsel the child for laboratory examination**

25. A two month old infant was brought to a health center with diarrhea of two days duration. On examination, the child was alert, skin pinch goes back quickly and drink fluid. A nurse noticed that there is no ORS to treat the child's condition at the health center.

What is the most likely intervention to treat this child? (Hidar 2012/ December 2019)

- A. Offer soft drink
- B. Give orange juice
- C. **Provide breast milk**
- D. Provide mixture of salt and sugar

26. A seven-year-old child was admitted with swelling of both foot, blood pressure of 150/90mmHg, PR: 130/min and gross hematuria. Accordingly, the child started treatment with frusemide.

What is the most likely indicator of this child's response to treatment? (Hidar 2012/ December 2019)

- A. Vital signs
- B. **Daily weight**
- C. Potassium level
- D. Daily urine output

27. A nine-year-old child has undergone tonsillectomy for recurrent tonsillitis.

What is the priority postoperative nursing intervention of this child? (Hidar 2012/ December 2019)

- A. **Place the child in a side lying**
- B. Gurgle the throat with H₂O₂
- C. Provide anti-pain
- D. Provide oxygen

28. A 10-year-old child was presented with frequent urination of one week duration. Since three days, she had fever and forced to pass her urine before reaching the toilet.

What is the actual diagnosis of this child? (Hidar 2012/ December 2019)

- A. Functional urinary incontinence
- B. Reflex urinary incontinence
- C. **Urge urinary incontinence**
- D. Stress urinary incontinence

29. A four-year-old child was presented with skin rash of three days duration. The rash initially started from the right face and rapidly covers both faces. The rash has pustule and honey- colored crusted plaque.

What is the most likely diagnosis of this child? (Hidar 2012/ December 2019)

- A. **Impetigo**
- B. Tinea corporis
- C. Atopic Eczema
- D. Contact dermatitis

30. A three-year-old child sustained scald burn of face of one hour duration. The child has pain and face swelling. On examination, the child has second degree burn of 4% total body surface area.

What is the most likely potential nursing diagnosis of the child? (Hidar 2012/ December 2019)

- A. High risk for chronic pain
- B. Risk for impaired skin integrity
- C. **High risk for fluid volume deficit**
- D. Risk ineffective airway clearance

31. A two-year-old child was presented with itching and erythematous skin of one week duration. The child skin is dry and becomes pale for a while after itching. The child has a similar family history.

What is the likely diagnosis of this child? (Hidar 2012/ December 2019)

- A. Scabies
- B. Tinea corporis
- C. **Atopic dermatitis**
- D. Contact dermatitis

32. A two-year-old child was presented with diarrhea of three weeks duration. On examination, the child refused to drink fluid and has dry mouth and lip. The child has a weight for height 66%.

What is the primary nursing intervention of the child? (Hidar 2012/ December 2019)

- A. Treat the child with IV fluid
- B. Treat the child with ORS solution
- C. **Treat the child with ReSoMaL fluid**
- D. Treat the child with Zink and Vitamin A

33. A 36-hour urine culture of a child that started treatment for UTI indicated no bacteria.

What does the treatment response of this patient indicate? (Hidar 2012/ December 2019)

- A. The child was mistakenly treated for UTI
- B. The child has totally cured from the disease
- C. The child has shown progress of healing
- D. Culture is not the appropriate test for UTI

34. A four-year-old child presented with painful, moist, red, and blistered scald burn. The wound has blanching.

What is the degrees of this child's burn? (Hidar 2012/ December 2019)

- A. First degree
- B. Second degree
- C. Third degree
- D. Fourth degree

35. A 12-year-old girl was admitted with fracture of left femur. Which is treated with bone traction.

What is the most likely actual nursing diagnosis for this child? (Hidar 2012/ December 2019)

- A. Activity intolerance
- B. Impaired skin integrity
- C. Impaired bed mobility
- D. Self-care deficit for feeding

36. A three-year-old child was brought to an emergency department with severe abdominal pain, frequent vomiting and bloody diarrhea. He was found playing with an insecticide container labeled with "high arsenic content" prior to the onset of signs.

What is the additional sign needed for the diagnosis of poisoning? (Hidar 2012/ December 2019)

- A. Dry mouth and thirst
- B. Acidotic-like breathing
- C. Red skin around the face
- D. Garlic odor on the breath

37. A two-year-old child was rushed to an emergency department with irritability and foaming of saliva after ingestion of tea cup full of laundry bleach that was placed by his mother for cleaning purposes.

What is the most likely emergency management of this child? (Hidar 2012/ December 2019)

- A. Inducing vomiting
 - B. Give milk or water
 - C. Give activated charcoal
 - D. Give nothing by mouth
38. The nurse evaluate a two-year-old child who was admitted for the treatment of failure to thrive secondary to severe acute malnutrition. The child had good appetite and no edema for 10 consecutive days. The nurse decided to discharge the child.

What is the most likely additional finding that could support the nurse's decision? (Tikimt 2013/October 2020)

- A. Weight for age >85%
- B. Weight for age >90%
- C. Weight for height >80%
- D. Weight for height >85%

Discharge criteria Or Page: 172

The child may be discharged if he/she fulfills the following criteria.

Weight /height $\geq$ 85%.

No edema for the last 10 days.

Complications are adequately treated.

Education completed.

mmunization up-to-date

39. A 15-month-old child was brought to health center for growth and development checkup. On examination, the infant can walk independently but cannot grasp of an object using thumbs and index finger. The child responds to a simple command like “wave bye bye“.

What was the most likely development problem of this child? (Tikimt 2013/October 2020)

- A. Emotional developmental problem
 - B. Language developmental problem
 - C. Cognitive developmental problem
 - D. Fine motor developmental problem
40. A nurse in immunization room found that the refrigerator is out of order. The technician of the health center confirmed that maintenance of the refrigerator could take a maximum of four to five days. So, the nurse wants to store vaccines till the refrigerator maintained.

What is the most likely type of cold chain used to store the vaccines in this time? (Tikimt 2013/October

2020)

- A. Cold box with cool water pack
- B. Cold box with frozen water pack
- C. Vaccine carrier with cool water pack
- D. Vaccine carrier with frozen water pack

41. A health center has a total of 20,000 population in the catchment area, of which infants account 20%. In this health center, the planned annual coverage for PCV vaccine is 90% with vaccine wastage multiplication factor of 1.11.

What is the annual maximum stock level of PCV vaccine in the health center? (Tikimt 2013/October 2020)

- A. 11,988 doses
 - B. 12,988 doses
 - C. 14,985 doses
 - D. 15,985 doses
-
- The calculation is as follows:
 - number of surviving infants (pt) = 200
 - number of pentavalent doses in the EPI schedule (dn) = 3
 - target coverage rate (ic) expressed as a decimal number = 0.9 (90%)
 - wastage factor (wf) = 1.33
 - Annual pentavalent vaccine needs = $pt \times dn \times ic \times wf$
$$= 200 \times 3 \times 0.9 \times 1.33$$
$$= 178$$
 - number of surviving infants (pt) = 4000
 - number of pentavalent doses in the EPI schedule (dn) = 3
 - target coverage rate (ic) expressed as a decimal number = 0.9 (90%)
 - wastage factor (wf) = 1.11

42. A nurse wants to initiate feeding for 1.5Kg neonate admitted in neonatal intensive care unit. The infant had a very weak sucking reflex and on risk for necrotized enterocolitis. (Tikimt 2013/October 2020)

What is the most appropriate initial step in this patient's care?

- A. Pouring the expressed breast milk via NG tube continuously as demanded
 - B. Inject 4.5ml expressed breast milk using 10ml syringe at a time per feeding
 - C. Give bolus 3ml expressed breast via NG tube over 20-25 min per feeding
 - D. Give bolus the expressed breast milk using 10ml syringe over 5-10min per feeding
-
43. A nurse assessed the progress of a two-year-old child who was on phase one management of severe acute malnutrition. The child was improved clinically with an evidence of reduced edema grade two and complication were completely treated.

What is the mostly likely additional finding used to proceed to the transition phase? (Tikimt 2013/October

2020)

- A. The edema has to be reduced to grade for a day
- B. The edema has to be lost completely for consecutive days
- C. Improved appetite evidenced by finishing F-75 food easily
- D. Improved appetite evidenced by finishing F-100 food easily

44. A 16-hour-old preterm neonate who was born at home was admitted to the NICU inability to feed and lethargy. On examination, the neonate had body temperature of 36.2°C, pulse rate 140/min and blood glucose level of 354mg/dl.

What is the best nursing intervention to manage the neonate's problem? (Tikimt 2013/October 2020)

- A. Provide intravenous glucose
- B. Put skin-to-skin with mother
- C. Provide milk through NG tube
- D. Warm the child using hot water bottle

Reason:

Management of neonatal hyperglycemia

- Treat the neonate if in shock or dehydrated accordingly
- Decrease glucose infusion by 2mg/kg/min (30ml/kg/day) every 6 hrs and stop gradually if on IV infusion.
- If the newborn was not on infusion, put him/her on 5% glucose with rate of 4mg/kg/min.(~ 60ml/kg/day).
- Look for and treat underlying causes.

45. A nurse who is giving immediate newborn care dries and stimulates the newborn. Unfortunately, the newborn do not cry.

What is the most appropriate next step for this newborn? (Tikimt 2013/October 2020)

- A. Suck fluid from the nose
 - B. **Bag and mask ventilation**
 - C. Observe and evaluate breathing
 - D. Back rubbing and flicking soles
46. A six-day-old neonate was admitted to a hospital for management of breathing difficulty. The neonate had spontaneous breathing at birth but breathing difficulty started on second day of birth and worsened on the fourth day. The neonate was put on Continuous Positive Airway Pressure Ventilation.

What is the best and safest method of checking the progress of this neonate? (Tikimt 2013/October 2020)

- A. **Pulse oximetry**
 - B. Blood gas analysis
 - C. Pulmonary function test
 - D. Carbon dioxide monitoring
47. A 15-day-old neonate was brought to a hospital with a complaint of difficulty of feeding of one day duration. The neonate had history of convulsion. On examination, the neonate had also trismus and irritability.

What is the most likely diagnosis of this neonate? (Tikimt 2013/October 2020)

- A. Sepsis
 - B. **Tetanus**
 - C. Meningitis
 - D. Hypoglycemia
48. A nurse was giving resuscitation for a newborn by using bag and mask ventilation. After a minute of resuscitation, the neonate didn't start spontaneous breathing and had pulse rate of 90/min.

What condition makes the nurse to stop resuscitation? (Tikimt 2013/October 2020)

- A. No spontaneous breathing after 10 minute
- B. Breathing less than 30 breath/min after 10 minute
- C. Breathing less than 30 breath/min after 20 minute
- D. **Pulse rate of less than 60 beats/min after 20 minute**

49. A 12-day-old neonate was brought to a hospital with a complaint of petechial rash all over the trunk for one day duration. On examination, the neonate had lowered platelet count, increased prothrombin time and increased partial thromboplastin time.

What additional information is the most likely used to identify the neonate's problem? (Tikimt 2013/October 2020)

- A. Neonatal history of bleeding
 - B. Maternal history of infection
 - C. Neonatal history of vitamin K injection at birth
 - D. **Maternal history of warfarin use during pregnancy**
50. A low birth weight neonate was brought to NICU with the clinical feature of unable to breast feed, difficulty of breathing and yellowish discoloration of a body part. The neonatal nurse classifies the neonate as jaundice according to IMNCI. What is the most likely finding used to confirm this classification? (Tikimt 2013/October 2020)
- A. Yellowish discoloration of palm and sole
 - B. Yellow skin and eye, and baby is six-day-old
 - C. Yellow skin and eye, and baby is 16-hour-old
 - D. **Yellow skin and eye, and baby is seven-day-old**
51. A four-year-old child was presented to the health center with diarrhea of five days duration. On physical examination, the skin pinch goes back within two seconds. And the child was very eager to drink water and had WFH z-score > -1.

What is the most appropriate management of this child according to IMNCI? (Tikimt 2013/October 2020)

- A. **Give ORS over 4-hours period**
 - B. Give ORS solution in each loss
 - C. Give ReSoMAL solution in each loss
 - D. Give RL solution intravascular over three hours
52. An eight-month-old infant was brought to a pediatric OPD with the clinical features of cough and **chest indrawing**. The respiration rate of the child was 48/min.

What is the most likely classification of the infant according to IMNCI? (Tikimt 2013/October 2020)

- A. **Pneumonia**
- B. Cough or cold
- C. Severe pneumonia
- D. Very severe disease

53. An infant who was born from a known HIV positive mother is brought to a pediatric OPD with the clinical features of cough, stridor and oral thrush. The HIV status of the infant was positive for antibody test and unknown for DNA/PCR test due to unavailability of the test.

What is the appropriate management for the child according to IMNCI? (Tikimt 2013/October 2020)

- A. Initiate PMTCT care
 - B. **Initiate ARV treatment**
 - C. Advise on HIV prevention
 - D. Advise to stop breastfeeding
54. A 30-month-old child was brought to the health center with a complaint of cough of three weeks duration. The nurse identified that the father of the child is on initiation phase of TB treatment and the child also had low grade fever.

What is the most appropriate initial step to do for this child? (Tikimt 2013/October 2020)

- A. Refer urgently to the hospital
 - B. Initiate INH as TB prophylaxis
 - C. **Put the child on anti-TB treatment**
 - D. Counsel on the diagnosis of TB exposure
55. A 10-month-old HIV exposed infant was brought to the under-five clinic for routine checkup. The nurse immediately took DBS sample and found DNA PCR test positive.

What is the most appropriate initial step the nurse should do? (Tikimt 2013/October 2020)

- A. Take DBS to repeat viral test
 - B. **Start ART as soon as possible**
 - C. Start cotrimoxazole immediately
 - D. Start ARV prophylaxis immediately
56. A four-year-old was brought to a health center with a complaint of cough of three days duration. On assessment, the nurse observed that the child had inspiratory stridor and barking cough.

What is the most likely diagnosis of this patient? (Tikimt 2013/October 2020)

- A. **Croup**
- B. Pneumonia
- C. Pulmonary TB
- D. Tonsil-pharyngitis

57. A child was brought to a pediatric OPD with the clinical feature of nasal congestion, sneezing, lacrimation and conjunctival suffusion. The nurse diagnosed the child has pertussis.

What is the most likely finding that could support the nurse's suspicion? (Tikimt 2013/October 2020)

- A. Dyspnea
- B. Rhinorrhea
- C. Low grade fever
- D. Whooping cough

58. A six-week-old infant was brought to pediatric OPD with the clinical features of high grade fever, neck stiffness, bulging fontanel and high pitch cry.

What is the most likely cause of this infant's problem? (Tikimt 2013/October 2020)

- A. Escherichia coli
- B. Group B streptococcus....most common for less than 2 months
- C. Group A streptococcus
- D. Staphylococcus aureus

59. A child was admitted to pediatric ward for the treatment of femoral bone fracture. The child was on traction with a cast and complained severe unpleasant sensation at the fracture site.

What is priority nursing diagnosis for this case? (Tikimt 2013/October 2020)

- A. Acute pain
- B. Impaired mobility
- C. Activity intolerance
- D. Impaired activities of daily living

60. A three-year-old child who was brought to a health center for vaccination aspirated coin while playing.

What is the next most appropriate nursing intervention to be done for this child? (Tikimt 2013/October 2020)

- A. Give artificial respiration
- B. Perform Heimlich maneuver....Abdominal Thrust maneuver
- C. Dislodge the coin with little finger
- D. Refer the child to nearby hospital urgently

61. A 20-day-old male baby was brought to pediatric OPD with a complaint of being unable to feed and convulsion. On examination his temperature was 38c, heart rate 98/min and CSF finding showed that his Leukocytes were 30/mm³, protein 110 mg/dL and 60% of the blood glucose.

What is the most likely diagnosis of this baby? (Tikimt 2013/October 2020)

- A. Neonatal seizure
 - B. Neonatal sepsis
 - C. Neonatal asphyxia
 - D. Neonatal meningitis
62. A six-year-old child came to pediatric emergency with a complaint of sudden loss of consciousness. The child was found to be in diabetic ketoacidosis. The nurse planned to administer fluid for the child. Child's weight was 16kg.

What is the most appropriate fluid to be given in the first hour of admission? (Tikimt 2013/October 2020)

- A. 160-320ml IV DNS
- B. 160-320ml IV 0.9% NS
- C. 160-320ml IV 0.45% NS
- D. 160-320ml IV 5% dextrose

DKA Treatment Protocol

TIME	THERAPY	COMMENTS
1st hour	10–20 mL/kg IV bolus 0.9% NaCl or LR	10 mL/kg IV bolus for mild DKA 20 mL/kg IV bolus for moderate to severe DKA
	Insulin drip at 0.05 to 0.10 µ /kg/hr	
2nd hour until DKA resolution	0.45% NaCl:plus continue insulin drip	Iv rate=85ml/kg+ Maintenance Fluid .-bolus for 23hrs
	20 mEq/L KPhos and 20 mEq/L KAc	If K < 3 mEq/L, give 0.5 to 1.0 mEq/kg as oral K solution OR increase IV K to 80 mEq/L
	5% glucose if blood sugar <250 mg/dL	

63. A seven-year-old patient came to pediatric OPD with a complaint of rapid, shock-like and non- rhythmic convulsion of four episodes with two days duration.

What is the most likely diagnosis of this child? (Tikimt 2013/October 2020)

- A. Clonic seizure
- B. Petitmal seizure
- C. Grandma seizure
- D. Myoclonic seizure

64. A child weighting 5kg was admitted to pediatric ward with a complaint of unable to feed. On examination, the baby had moderate respiratory distress and his hematocrit level was 25% (HCT packed RBC to be given was 70%).

What is the amount of packed RBC to be transfused for this patient? (Tikimt 2013/October 2020)

- A. 160 ml
- B. 182 ml
- C. 198 ml
- D. 204 ml

Volume of packed red blood cell to be transfused can be calculated as follows.

$$\text{Volume of transfusion} = \frac{\text{Weight in Kg} \times \text{blood volume} / \text{Kg} (\text{desired HCT} - \text{observed HCT})}{\text{HCT of blood to be given}}$$

- Average HCT of packed RBC is in the range 70-80%
- Transfuse over 2-4 hours time

65. An adolescent girl came to pediatric clinic and diagnosed with uncomplicated nephrotic syndrome.

What is the most appropriate nursing intervention for this patient? (Tikimt 2013/October 2020)

- A. **Administration of prednisone**
- B. Administration of cyclosporine
- C. Administration of cephalosporin
- D. Administration of cyclophosphamide

66. A two-year-old female child was brought to a health center with difficulty of feeding and fever of three days duration. On examination, she has rash around the face and neck and the clinician suspected measles.

What could be the pathognomonic sign to substantiate this diagnosis? (Tikimt 2013/October 2020)

- A. Red eyes
- B. Rose spots
- C. Runny nose
- D. **Koplik spots**

67. An infant with confirmed hemophilia B developed hematuria and admitted to the hospital. The nurse provided the appropriate amount of maintenance fluid with bed rest but hematuria continued for consecutive days.

What is the most appropriate next step in the management of this infant? (Tikimt 2013/October 2020)

- A. **Give 20IU/kg factor IX**
- B. Give 40IU/kg factor IX
- C. Give 40IU/Kg factor VIII
- D. Give 20IU/Kg factor VIII

In the case of an infant with hemophilia B and persistent hematuria, the most appropriate next step in management is to give factor IX replacement therapy. The standard dosing for factor IX in cases of bleeding is typically around 20-40 IU/kg. Given the options provided, the higher dose would be more appropriate in managing significant bleeding.

68. A 12-year-old male child who came to town by a child trafficker frequently asked for help. During the interview, the child claims that he does not have any relative and frequently abused by the gangs in the street.

What is the most likely priority intervention for the child? (Tahisas 2014/ December 2021)

- A. **Look for possibilities to reunite with his families**
- B. Reassure the child about availability of health facility
- C. Attach the child to credible children home care providers
- D. Provide street based health education on personal hygiene

69. A three-month-old child came with his mother to under-five children OPD with the complaint of diarrhea for three days. During history taking, the mother claims that “this disease might associate with my practice of giving only breast milk and also he cried a lot now. I think the milk is not enough, it is better to start giving a complementary food.” Then, the nurse wants to advice a woman about breast feeding.

What is most likely feeding advice for this mother? (2014/2022)

- A. Continue breast feeding until 2 years
- B. Use good positioning and attachment technique
- C. Continue exclusive breastfeeding until six months
- D. Have to empty one breast before switching to the other

70. A four-year-old child came to a health facility with a chief complaint of abdominal pain and irritability followed by fatigue, anorexia, anemic manifestation, gait disturbance, and slurred speech. During history taking, the mother claims that the baby was frequently playing using wires and metallic objects.

What is the most likely cause of poisoning to this child? (2014/2022)

- A. Lead
- B. Arsenic
- C. Cyanide
- D. Mercury

71. A four-year-old child was brought to pediatric OPD for follow up care. The child was on supplementary feeding in the community based management of acute malnutrition. On examination, child's weight-for-height was 65% and no other co-morbidity.

What would be the interpretation of the anthropometric measurement of this child? (2014/2022)

- A. Mild acute malnutrition
- B. Severe acute malnutrition
- C. Severe chronic malnutrition
- D. Moderate acute malnutrition

72. A premature neonate was admitted to neonatal intensive care unit for the treatment of low birth weight. The neonates has weak suckling reflex, absence of grasping and tonic neck reflexes.

What is the priority nursing diagnosis for this case?

- A. Altered growth pattern
- B. Ineffective breast feeding
- C. Altered development pattern
- D. Altered neuro-muscular function

73. A primary health care unit has a total of 11,000 population in the catchment area, of which 20% were infants. The PHCU was planning to know the annual pentavalent vaccine consumption. The expected annual coverage is 90% with wastage factor of 1.05.

What will be an estimated annual need of pentavalent vaccine for this PHCU?

- A. 1980 doses
- B. 2200 doses
- C. 2079 doses
- D. 6237 doses

Estimated Annual Need of Pentavalent Vaccine

To calculate the estimated annual need for the pentavalent vaccine:

Total Population: 11,000

Infants (20%): $11,000 \times 0.20 = 2,200$ infants

Expected Coverage (90%): $2,200 \times 0.90 = 1,980$ doses

Wastage Factor (1.05): $1,980 \times 1.05 = 2,079$ doses

Thus, the estimated annual need of pentavalent vaccine for this PHCU is:

74. The nurse found inappropriate arrangement of vaccine cold box at a health center. The temperature-monitoring device indicated 18°C. The nurse assessed the vaccine vial monitor to check whether the vaccines were damaged or not.

What would be the most likely finding used as a discarding point?

- A. The inner square of the VVM was lighter than the outer circle
- B. The inner square of the VVM was highly darker than the outer circle
- C. The inner square of the VVM was slightly lighter than the outer circle
- D. The inner square of the VVM was similar in color with the outer circle

75. A nurse has been observing a lactating mother on baby positioning during breast feeding. On observation, the infant faces to the mother, the whole body is supported and touches mother's abdomen.

What is the most likely additional finding used to confirm good positioning?

- A. Chin of the infant should touch the breast
- B. The infant's mouth should be widely open
- C. More areola seen on the upper part of breast
- D. Head and body of the infant should be straight

76. A pediatric patient, who was on nutritional rehabilitation for marasmus management, developed vomiting,

diarrhea and hypothermia. On examination, the nurse suspected that the child has developed dehydration as a complication of severe acute malnutrition.

What would be the most likely clinical sign that supports the nurse's suspicion?

- A. A history of sunken eye
 - B. Skin pinch goes very slowly
 - C. Lethargy for level of consciousness
 - D. Weak or absence of peripheral pulse
77. A four-day-old neonate was brought to a hospital with red and swollen eyes of two days duration. On examination, the neonate has draining pus from the eyes.

What is the most appropriate initial nursing intervention to this neonate?

- A. Apply 1% tetracycline eye ointment
- B. Administer ceftriaxone 50 mg/kg IM stat
- C. Clean the eyelids using sterile normal saline
- D. Treat the neonate with stat dose of ciprofloxacin

78. A very preterm newborn had non breathing at birth. A nurse started drying and rubbing its back with dry towel but the newborn had no spontaneous breathing.

What is the next appropriate nursing care for this neonate?

- A. Clamp and cut the cord
- B. Suck fluid from the nose
- C. Bag and mask ventilation
- D. Administer low pressure oxygen

79. A seven-day-old preterm neonate was treated for stage three necrotizing enterocolitis with antibiotics and was kept NPO. The nurse wants to assess the progress of this neonate after two days of treatment.

What is the most appropriate sign the nurse should observe to confirm good progress?

- A. Soft and non-tender abdomen
- B. Passing loose greenish stools
- C. Ability of the neonate to feed breast milk
- D. Absence of pneumatosis intestinalis on radiograph

80. A term newborn was found to be jaundiced and unable to suck breast milk within the first week after birth. A nurse plans to put the neonate under phototherapy.

What is the most appropriate next step the nurse should do?

- A. Adjust the light of phototherapy
- B. Secure intravenous line for feeding
- C. Obtain the result of serum bilirubin level
- D. Cover the eye of the neonate with plaster

81. A neonate was born at 34 weeks of gestational age and weighed 1,900gm at birth. On assessment, the neonate had body temperature of 36.7°C at one hour of birth.

What is the most likely nursing intervention for this neonate?

- A. Admit the neonate for two days
- B. Appoint the mother for two days follow up
- C. Give the neonate polio zero and BCG vaccine
- D. Tell the mother to wash the neonate after 24 hour

82. An 18-month-old child with the complaint of ear pain and drainage of pus from the ear for the last eight days brought to under-five OPD/

What is the appropriate management for this child according to IMNCI?

- A. **Treat with amoxicillin for 5 days**
- B. Give first dose of ampicillin and refer
- C. Give first dose chloramphenicol
- D. Treat with quinolone ear drop for 2 weeks

83. A nine-month-old infant with the complaints of cough and intermittent vomiting was brought to a health center. The nurse classified the patient as very severe disease according to IMNCI.

What is the most likely finding to confirm this classification?

- A. Stiffness of neck
- B. Bulging fontanel
- C. **History of convulsion**
- D. Weak sucking of breast

84. A known HIV positive child with the complaints of cough and fever for two weeks was brought to a health center. The child has history of contact with non MDR TB patient from the parent in the last one year.

What is the most likely TB status classification of the child according to IMNCI?

- A. TB infection
- B. **Suspected TB... TB Disease**
- C. No TB infection
- D. TB exposed

85. A three-year-old child, who was on treatment for uncomplicated severe acute malnutrition came back after seven days of treatment for follow up care.

What should the nurse primarily check for this child according to IMNCI?

- A. **Appetite**
- B. MUAC
- C. WFH z-score
- D. Danger signs

86. A 17-year-old child, who was on anti-TB for five weeks, became very sick. On laboratory investigation, child's WBC is $8 \times 10^3/m^3$. The nurse suspected drug toxicity and decided to stop the drug.

What is the most likely finding that supports the decision of the nurse?

- A. Presence of orange/red urine
- B. Presence of visual impairment
- C. Presence of anorexia and/or nausea
- D. Presence of burning sensation in feet

87. A seven-month-old HIV exposed infant was brought to the health center with a complaint of fever for ten days duration. On examination, the child had severe pneumonia and oral thrush. The child was positive for HIV antibody test and DNA PCR test was not available.

What is the most appropriate intervention for the child?

- A. Give anti-fungal as a symptomatic management
- B. Enroll mother of the baby in cohort follow up at PMTCT clinic
- C. Treat pneumonia with PO amoxicillin and send back to home
- D. Put the child on ART for presumptive severe HIV disease

88. A two-year-old child was brought to pediatric OPD with the complaint of abdominal cramp, fever and bloody diarrhea.

What is the most likely cause of this child's problem?

- A. Shigella
- B. Vibrio cholera
- C. Gardia labmlia
- D. Salmonella species

89. A child was admitted to pediatric ward for the treatment of acute gastroenteritis. A day after admission, the child complained unpleasant, wave-like sensation in the epigastric area and throughout abdomen that leads to vomiting.

What is the most likely nursing diagnosis of the child?

- A. Nausea
- B. Anorexia
- C. Impaired swallow
- D. Imbalance nutrition

90. An adolescent came to a clinic with the A five-year-old child was brought to a pediatric OPD with the clinical feature of oliguria. Rapid weak pulse, hematuria, proteinuria and generalized body edema.

What is the priority nursing diagnosis of the child?

- A. Urinary retention
- B. Fluid volume excess
- C. Decrease cardiac output
- D. Impaired tissue perfusion

91. A two-year-old baby was admitted to pediatric ward with a diagnosis of septic arthritis. The nurse planned for an intervention that would enhance wound healing.

What is the priority intervention the nurse should give?

A. Debride of infected wound

- B. Administer high protein diet
- C. Administer systemic antibiotic
- D. Instill antibiotics to the affected joint

92. complaints of warmth, erythema and tenderness. On examination, the lateral margins tended to be indistinct and there was pitting edema. The clinician wants to manage this problem.

What is the most important parameter for the empirical therapy of this patient? (2014/2022)

- A. Age
- B. Sex
- C. Height
- D. Weight

93. A four-year-old child was brought to pediatric emergency with a complaint of breathing difficulty. On examination, the nurse suspected foreign body aspiration.

What is the most important clinical feature the nurse should check to confirm this diagnosis? (2014/2022)

- A. Gagging
- B. Atelectasis
- C. Hemoptysis
- D. Sign of infection

94. An eight-year-old child was admitted to pediatric ward with a diagnosis of burn injury. The nurse planned to administer tetanus vaccine for this child.

What was the most likely finding of the nurse to make this decision? (2014/2022)

- A. Scald burn
- B. Inhalation burn
- C. Electrical injuries
- D. Surface area of burn

95. A school-age child came to pediatric OPD with the manifestation of acute, rapidly progressive and potentially fulminating course of high grade fever, dyspnea, drooling and rapidly progressing respiratory obstruction. On history; the child did not take H.influenza type b vaccine.

What is the most likely diagnosis of this child? (2014/2022)

- A. Croup
- B. Laryngitis
- C. Pneumonia
- D. Epiglottitis

96. A 10-year-old patient was brought to a health center with coma. On laboratory investigation, *P.falciparum* was found from the blood film result.

What is the first line drug to be given for this patient? (2014/2022)

- A. Quinidine
 - B. Artemether
 - C. Artesunate
 - D. Quinine dihydrochloride
97. A 30-year-old woman came to a health center, from an area where goiter is endemic (prevalence >15%), for the first ANC follow up. On laboratory investigation, the nurse found decreased level of iodine in the maternal blood.

What is the most appropriate measure to prevent fetal hypothyroidism? (2014/2022)

- A. Advise the mother to feed sea foods
 - B. Counsel the mother to feed iodized salt
 - C. Give IM depot injection of iodized oil
 - D. Advise the mother to avoid goiterogenic nutrients
98. A pediatric patient with the complaints of diarrhea, gaseous abdominal distention, and failure to gain weight came to the health center. The nurse suspected carbohydrate mal-absorption disorder.

What is the most important additional finding that could substantiate the nurse's suspicion? (2014/2022)

- A. Basic stool using PH
 - B. Edema of lower extremities
 - C. Increased nitrogen gas in the breath
 - D. Increased hydrogen gas in the breath
99. A two-year-old child was brought to a hospital with a complaint of cough of two days duration. The child has poor feeding, inspiration stridor, and convulsion during examination and nasal flaring. The child's respiratory rate is 50/min, pulse rate is 110/min and temperature is 37.5°C.

What is the most appropriate nursing management for this child? (2014/2022)

- A. Administer 0.5 mg/kg Diazepam injection solution per rectum
- B. Administer first dose of IV/IM Ampicillin and Gentamycin
- C. Administer oral Amoxicillin for five days
- D. Administer maintenance fluid IV

100. A two-year-old child was brought to a hospital with a complaint of diarrhea of three days duration. The diarrhea is watery, but not bloody. The child has sunken eyes and was not able to drink, skin pinch goes back within four seconds, and weight for length z-score is < -3 standard deviation.

What is the most appropriate drug management for this child? (2014/2022)

- A. Administer 75 ml/kg ORS for four hours
 - B. Administer 5 ml/kg 5% glucose solution for two hours
 - C. Administer 5 ml/kg ReSoMal solution per 30 minutes two hours
 - D. Administer 100 ml/kg IV Ringer's lactate solution for three hours
101. A HIV positive woman gave a birth for her first baby. She was not on follow up and the infant was exposed to HIV which was identified through infant HIV antibody testing. The infant is on breastfeeding and the nurse has anticipated the case and intended to give proper care to prevent the infant from actual infection.

What is the most appropriate action that the nurse should take in this case? (2014/2022)

- A. Provide Abacabir prophylaxis for the baby
 - B. Provide Nevirapine prophylaxis for six week
 - C. Recommend mother to stop breast feeding immediately
 - D. Provide Stavudine prophylaxis for the baby for six month
102. A nurse in the PMTCT unit has prepared a nursing care plan to provide a holistic care to HIV exposed infants. A mother planned to stop breast-feeding today following the advice from the nurse. On the follow up visit schedule, the nurse planned to do a rapid antibody test.

When is the best time to do the antibody test in this case scenario? (2014/2022)

- A. At birth
 - B. At 6 weeks
 - C. At 10 weeks
 - D. At 12 weeks
103. A three-year-old child has been admitted due to severe acute malnutrition. The child has cough of six days duration. Physical examination during admission shows that the child has bilateral pitting edema (++), bulged abdomen, and skin lesion (++). The child has been on treatment for severe acute malnutrition. At six day of admission, the child has, good appetite, edema decreased (+) with no IV or NG tube.

What will be the progress of a child based on the current situation? (2014/2022)

- A. Should stay in phase one
 - B. **Transfer him to transition phase**
 - C. Based on the progress should be phase two
 - D. Transfer him to outpatient therapeutic program
104. A child has been brought to a hospital with a complaint of diarrhea for five days duration. The nurse wants to assess the growth and development status of the child. The anthropometric result shows weight for height < -1 standard deviation, mid upper arm circumference (MUAC) is 12 cm, height for age is < -2 standard deviation and weight for age is < -1 standard deviation.
- What is the most appropriate interpretation of the child's anthropometric measurement? (2014/2022)
- A. Under weight
 - B. Moderate wasting
 - C. **Acute malnutrition**
 - D. Chronic malnutrition
105. An 18-month-old child, who was brought to a hospital for growth checkup, was assessed for development status. The nurse has identified that the child can walk alone, crawls up stairs, makes tower of 4 cubes, speaks an average 10 words, and can feed himself. From assessment, the nurse has noticed that the child has red flags of under development.
- What is the most likely delayed development domain noticed by the nurse? (2014/2022)
- A. **OPV**
 - B. Measles
 - C. Penta
 - D. Rotavirus
106. A nurse had been assessing a mother on breast-feeding clues for adequate breast-feeding. On observation, the infant sucked five times before pausing and gurgle sound was heard during swallowing.
- What is the most important additional finding that can show the adequacy of breast-feeding? (2014/2022)
- A. Chin of the infant should touch the breast
 - B. The infant's mouth should be widely open
 - C. **More areola seen on the upper part of breast**
 - D. Dimpling of cheeks may be seen while suckling
107. A 10-month-old child with clinical feature of severe wasting, edema of lower extremities and palmar pallor came to hospital. The nurse admitted and put the child on phase one management of severe acute malnutrition.

What is the priority nursing intervention for the child? (2014/2022)

- A. Iron folate
- B. Deworming
- C. **IV fluid infusion**
- D. Folic acid supplement

108. The child was complicated severe acute malnutrition was on treatment in the inpatient treatment program of severe acute malnutrition. On admission, the child had severe wasting, pitting edema and failed the appetite test. However, the child did not respond to treatment and classified under failure to treatment. What is the most likely indicator of failure to treatment the nurse assessed? (2014/2022)

- A. Failure to loss edema on day 10
- B. Failure to regain appetite on day 10
- C. If edema persists for more than 10 day
- D. **Failure to gain at least 5 g/kg bodyweight in day four**

109. A seven-year-old child was brought to a clinic after thermal burn on the chest, abdomen and left thigh. On arrival, the health care team started fluid admiration, cleaning and dressing the wound. However, the child suddenly develop shortness of breathing, edema on the lower leg and rapid and weak pulse. What could be the immediate nursing management for this child? (2014/2022)

- A. **Stop IV fluid**
- B. Increase IV fluid flow
- C. Put on 100% oxygen
- D. Relieve the pain using ice pack

110. A two-day-old neonate was brought to a hospital with a complaint of unable to breast- feeding of one-day duration. On physical examination, the neonate has brown eyes, yellowish palms and bluish mouth and lips. All neonatal reflexes were active. The nurse has suspected hyperbilirubinemia.

Which finding is the most confirmatory manifestation for this diagnosis? (2014/2022)

- A. **Yellow palms**
- B. Bluish mouth
- C. Brown eyes
- D. Bluish lips

111. A five-year-old child came to a hospital with a complaint of diarrhea of three days duration. The diarrhea is not bloody, but it is watery, rice water color and has no mucus.

What is the most likely cause of diarrhea in this case? (2014/2022)

- A. Salmonella species

- B. **Vibrio cholera**
- C. Rota virus
- D. Giardiasis

112. A **known HIV positive mother** brought her six-month-old infant to a hospital for growth monitoring and checkup. The baby was on exclusive breast-feeding. The nurse assessed the infant and there was poor weight gain. The child's antibody test was positive at six months of age and the DNA PCR test was pending. What is the primary indicator to say HIV exposed child in this case? (2014/2022)

- A. Poor weight gain
- B. Positive antibody test
- C. Exclusive breast feeding
- D. **Born from positive mother**

113. A two-month-old infant was brought to a hospital with a complaint of cough of two days duration. The infant is alert and can feed breast milk. The infant respiration is **50/min**, pulse rate is 110/min and temperature is 38°C. The nurse wants to classify this illness based on IMNCI protocol.

What is the most likely diagnosis of the child? (2014/2022)

- A. Very severe pneumonia
- B. Severe pneumonia
- C. **Pneumonia**
- D. Cough/cold